A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Zoe Brown
Melbourne Rheumatology Group

Dear Dr Brown,

Re: Ainslie Knight
DOB: 16/04/1993 Phone: 0406 270 828
Address: 206 / 24-26 Merri Street, BRUNSWICK, VIC, 3056
Email: ainsliecek@gmail.com

Summary

- **Significant improvement in bowel habit and reduction in bloating severity.**
- **Successful implementation of smoking cessation and dietary fibre increase.**
- **Commenced pelvic floor physiotherapy for pelvic floor dysfunction.**
- **Nortriptyline restarted at 10mg with a plan for slow up-titration.**
- **No action required by you**

I had the pleasure of reviewing Ainslie today. I am pleased to report that she has seen a significant improvement in her gastrointestinal symptoms since our last consultation. Most notably, she reported having a normal, solid bowel movement this week, which represents a major shift in her clinical baseline.

Ainslie has been very diligent with our recommended lifestyle interventions. She has successfully ceased smoking and has significantly reduced her intake of non-sugar sweeteners. She has also transitioned to a higher-fibre diet. Furthermore, she has commenced pelvic floor physiotherapy with Izzy. It is clear that pelvic floor dysfunction is a major contributor to her presentation; she is currently working on belly breathing and reverse Kegel exercises, and she already describes this as a "game changer" for her symptoms.

Regarding her bowel habit, her frequency has decreased from upwards of six times daily to approximately two to three times. While bloating remains a concern, it is now less frequent and less severe. She did mention experiencing some vaginal bleeding specifically when passing solid stools, which we suspect may be related to muscle guarding or pelvic floor tension. She has had a modest weight loss of approximately 2kg, but her appetite is stable and her previous symptoms of vomiting have resolved.

Impression and Plan

The primary challenge recently was a transient episode of numbness in her toes shortly after starting nortriptyline. She self-ceased the medication for a month, but has since restarted 10mg and has been stable for three weeks without recurrence of the paraesthesia. Ainslie reports some manageable dry mouth and some mood lability, though the latter is confounded by her recent cessation of the progesterone-only pill and her known history of ADHD.

I have advised Ainslie to remain on 10mg of nortriptyline for one further month to ensure no neurological symptoms recur. If she remains stable, she will then increase the dose to 20mg. We have also agreed to a

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trial of cessation of her laxatives, as her recent "overflow" symptoms may have been exacerbated by the medication itself. She will continue her intensive pelvic floor physiotherapy. I have provided a new prescription for nortriptyline and will review her again in four months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Louisa Martin, Ms Isabella Lees-Trinca

All appointments and enquires
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Endoscopy locations
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320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065